INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Docetaxel 75 mg/m² · every 3 weeks

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00203
Version 7

> Docetaxel 75 mg/m² by intravenous (IV) infusion on Day 1 every 21 days. For the source breast indication, continue until progression or unacceptable toxicity; the team reviews benefit and tolerability. The six-cycle source maximum applies to non-small-cell lung cancer (NSCLC).

Treatment stages

01 **Docetaxel**
21-day interval

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Docetaxel 75 mg/m² · every 3 weeks
National Cancer Control Programme (NCCP) 00203

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathing symptoms, severe diarrhoea, inability to drink, painful swollen limbs, unusual bleeding, or worsening numbness. Tell staff immediately about infusion-site pain or an allergic reaction.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How the medicines work

The aim and duration depend on the primary cancer and exact NCCP indication. Confirm the cancer-specific plan before treatment; this shared medicine schedule does not replace that discussion.

TAXANE CHEMOTHERAPY

Docetaxel



Disrupts the cell’s division machinery

Docetaxel stabilises tiny structures called microtubules. Cells need these to separate their chromosomes during division; disrupting the process can stop cancer cells multiplying. Healthy dividing cells are also affected.

Microtubules stabilised → Division disrupted → Cell growth affected

What to watch for

Take prescribed steroid premedication. Report fever, lasting numbness, swelling, sore nails or infusion symptoms.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim

Indication

Source-defined breast, lung, gastric, gynaecological and castration-sensitive prostate cancer (including high-risk nonmetastatic prostate disease)

Schedule and phase changes

Docetaxel 75 mg/m² by intravenous (IV) infusion on Day 1 every 21 days. Duration depends on the cancer and treatment aim: non-small-cell lung cancer (NSCLC) has a source maximum of six cycles; other indications require an explicitly agreed course, including prostate treatment. This differs from the 100 mg/m² card.

Evidence in context

Dose-specific breast evidence; no numeric result is transferred across dose arms or to the other tumour indications in 00203.

Why this treatment may be chosen

Docetaxel may slow or shrink advanced breast cancer and help symptoms. The dose is selected to balance benefit and side effects; results from a different dose or another cancer do not predict your outcome.

The treating team considers the cancer, previous treatment, health and your priorities. Trial averages do not predict an individual outcome.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Docetaxel 75 mg/m² · every 3 weeks
NCCP 00203

The phases and how they connect

Docetaxel

21-day view



Chemotherapy · Day 1
Day 1

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

21-DAY INTERVAL

Docetaxel

Repeated cycles

Docetaxel 75 mg/m² by intravenous (IV) infusion on Day 1 every 21 days. Duration depends on the cancer and treatment aim: non-small-cell lung cancer (NSCLC) has a source maximum of six cycles; other indications require an explicitly agreed course, including prostate treatment. This differs from the 100 mg/m² card.

Each phase has its own medicines, checks and supportive prescription. A change of phase does not restart the cardiac monitoring clock.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Milestones show order, not equal time intervals or booked appointments. The treating team requests tests, reviews results and confirms treatment. full blood count (FBC), renal and liver profile before each 21-day cycle; review infection, neuropathy, oedema, respiratory symptoms and steroid premedication before each infusion. NCCP breast treatment continues until progression or unacceptable toxicity. The source six-cycle maximum applies to NSCLC, not automatically to breast cancer.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00203 v7 · official regimen · EMA Taxotere product information

THE WHOLE TREATMENT JOURNEY

Docetaxel 75 mg/m² · every 3 weeks: treatment and checks

Docetaxel 75 mg/m² by IV infusion on Day 1 every 21 days. For the source breast indication, continue until progression or unacceptable toxicity; the team reviews benefit and tolerability. The six-cycle source maximum applies to NSCLC.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Treatment plan NCCP 00203	Confirm indication, goal and current prescription	Repeat 21-day cycles while beneficial and tolerated	Continue according to response, tolerance and prescribed course	Team review before restart or dose change	Agree completion or next treatment
Blood / kidney / liver NCCP 00203 · SMPC	Baseline FBC, kidney and liver tests	FBC, kidney and liver tests before each 21-day cycle	FBC, kidney and liver tests before each 21-day cycle	Recheck recovery and dose before restarting	Follow unresolved abnormalities
Symptoms / medicines NCCP 00203 · SMPC	Review symptoms, medicines and previous treatment	Review effects before each dose	Continue symptom and medicine reviews	Report problems; do not change or restart doses yourself	Agree recovery and next treatment plan
Heart / electrocardiogram (ECG) / echo NCCP 00203 · SMPC	Assess cardiac history and new symptoms; this docetaxel monotherapy source has no fixed serial echo/ multigated acquisition (heart pumping) scan (MUGA) schedule.	Review symptoms and indicated tests	Continue the risk- and treatment-specific plan	Urgent assessment for chest pain, fainting or breathlessness	Agree follow-up from exposure and results
Response / course NCCP 00203 · SMPC	Confirm cancer-specific treatment aim	Repeat 21-day cycles while beneficial and tolerated	Review benefit, symptoms and the agreed scan plan	Reassess if treatment is held or symptoms change	Agree continuation, change or completion
Bone health / other care NCCP 00203 · SMPC	Review previous/ ongoing treatments and fracture risk	No automatic chemotherapy -cycle dual-energy X-ray absorptiometry (bone density) (DXA)	Keep any separate endocrine/ fracture-risk plan	Review if clinical risks change	Continue separately indicated follow-up

What each check is for

1



Treatment and safety review

Before each planned treatment or four-week review for olaparib

FBC, renal and liver profile before each 21-day cycle; review infection, neuropathy, oedema, respiratory symptoms and steroid premedication before each infusion.

Ask / act: The treating team confirms results and the plan; this map does not authorise a dose.

2

Benefit and continuation

At agreed clinical review points

Docetaxel may slow or shrink advanced breast cancer and help symptoms. The dose is selected to balance benefit and side effects; results from a different dose or another cancer do not predict your outcome.

Ask / act: Discuss benefit, toxicity and priorities before continuing or changing treatment.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan



STEP 1

Before the first visit

Blood tests, relevant cardiac assessment, medicines review, fertility discussion and the consent conversation come first.



STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Staff check the line, give prescribed premedicines and treatment, and observe for reactions. Tell them immediately about burning, pain, rash or breathlessness.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

First visits can take longer because of loading doses, premedicines and observation. Ask the unit about expected time, transport and any escort needed after sedation or line insertion.

Phase-specific visit pattern

Infusion and observation times differ by drug and first versus later doses; the unit confirms the full visit duration, including blood review and preparation.

Checks through the course

A named team member should arrange and review each test.

Blood tests

FBC, renal and liver profile before each treatment dose

Heart tests

No routine fixed-cycle echo or ECG is mandated for this regimen alone; assess symptoms and individual cardiac risk. At a change or completion, agree cardiac follow-up from treatment exposure, symptoms and results.

Cancer imaging

Agree response imaging and any tumour markers for the actual primary cancer, disease sites and treatment aim. The metastatic breast 2–4-month example is not a universal interval for lung, prostate, gastric, ovarian or bladder cancer.

The wider cancer-specific plan

This medicine schedule serves several cancers; confirm which NCCP indication applies.

Local treatments

Surgery, radiotherapy and any endocrine / androgen-deprivation treatment follow the actual primary cancer and treatment aim. No breast surgery or HER2 phase is automatically added.

Bone health

Bone-health / dual-energy X-ray absorptiometry (bone density) (DEXA) requirements depend on endocrine or androgen-deprivation treatment, fracture risk and the primary cancer plan; not on this taxane alone.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Docetaxel

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Docetaxel

Treatment or check	Timing in this view
Chemotherapy · Day 1	 **Day 1** IV
Administration reaction	 **Days 1–21** Report during or after any visit
Nausea / appetite	 **Days 1–5** May occur early or later
Blood counts / infection	 **Days 1–21** Risk varies; fever needs prompt advice
Fatigue / mouth changes	 **Days 1–21** Variable timing and severity
Nerves / skin / nails	 **Days 1–21** Can build across doses

No dates to enter: The chart explains the pattern. Your team books actual appointments and reviews results.

Phase changes: Do not change the schedule using this educational map.

Treatment delays: A delayed dose does not automatically delay an echo or other test that is already due.

> **Why these bands stay broad:** These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00203 v7 · official regimen ↗

There are no guaranteed "safe days". Fever, shivers, sudden illness, severe breathing changes or chest pain need prompt assessment on any day.

Between-treatment notes

Day 1–21: Your current reference cycle

Docetaxel 75 mg/m² by IV infusion on Day 1 every 21 days. Duration depends on the cancer and treatment aim: NSCLC has a source maximum of six cycles; other indications require an explicitly agreed course, including prostate treatment. This differs from the 100 mg/m² card. Follow the team's appointment and medicine instructions; contact them about missed doses or new symptoms.

Support around the medicines

1 The correct phase prescription

Taxanes use low-risk antiemetic support; hypersensitivity premedication is a separate requirement.

2 Premedicines and growth factors

Docetaxel dexamethasone is taken for three days starting the day before treatment. Ask the team to reconcile this with antiemetic steroids.

3 Tests and appointments

The team should name the booking owner, due window and person reviewing each required test. Tell them if an expected appointment has not arrived.

4 After chemotherapy

Discuss recovery, fertility, rehabilitation and the next treatment or surveillance step.

5 Review the full medicine plan

Dexamethasone 8 mg orally twice daily for 3 days starting the day before docetaxel; at least 3 doses before treatment unless contraindicated. Missed doses require contact; 20 mg IV replacement is consultant-directed. Reconcile the complete steroid plan with the antiemetic plan; do not add duplicate dexamethasone. Low emetic risk: separate prevention/rescue medicines according to the national/local plan. Prophylactic granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) may be used after individual risk assessment.

Benefit, course and continuing checks

Heart checks and the full course

Assess cardiac history and new symptoms; this docetaxel monotherapy source has no fixed serial echo/MUGA schedule. NCCP breast treatment continues until progression or unacceptable toxicity. The source six-cycle maximum applies to NSCLC, not automatically to breast cancer.

Questions worth asking

When are heart checks due?

No routine fixed-cycle echo or ECG is mandated for this regimen alone; assess symptoms and individual cardiac risk. Test timing follows the treatment, clinical findings and local pathway.

Where do surgery and radiotherapy fit?

Surgery and radiation follow your actual cancer-specific treatment plan. This shared taxane card does not impose an adjuvant breast sequence.

Does a port mean a pump at home?

No. This regimen has no routine take-home infusion pump. A port is an access device; the team removes the needle after the visit when appropriate and arranges device removal separately.

Try explaining the plan back: Which phase are you in, what changes next, and who is arranging and reviewing your next blood test, echo or scan?

Docetaxel

- Low blood counts, infection and sepsis
- Hair loss, mouth soreness and fatigue
- Numbness / tingling; sometimes lasting
- Fluid retention and nail changes
- Allergic infusion reaction

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Docetaxel 75 mg/m² · every 3 weeks · reviewed supportive care

Active medicines: Docetaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed

Medicine	Dose / route	When
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- Docetaxel: dexamethasone 8 mg orally twice daily for 3 days starting the day before chemotherapy; at least 3 doses before the infusion. If missed, contact the team; any intravenous (IV) replacement is a consultant decision.
- Reconcile the total dexamethasone schedule with antiemetics, including aprepitant interactions. The listed antiemetic steroid is a reference row, not an instruction to add a second course.
- Assess granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) according to the exact regimen and individual risk.
- Dexamethasone 8 mg orally twice daily for 3 days starting the day before docetaxel; at least 3 doses before treatment unless contraindicated. Missed doses require contact; 20 mg IV replacement is consultant-directed.
- Reconcile the complete steroid plan with the antiemetic plan; do not add duplicate dexamethasone.
- Low emetic risk: separate prevention/rescue medicines according to the national/local plan. Prophylactic G-CSF may be used after individual risk assessment.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Docetaxel 75 mg/m² · every 3 weeks

Docetaxel 75 mg/m² by intravenous (IV) infusion on Day 1 every 21 days. For the source breast indication, continue until progression or unacceptable toxicity; the team reviews benefit and tolerability. The six-cycle source maximum applies to non-small-cell lung cancer (NSCLC).

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hands / feet

Docetaxel

Report numbness or tingling before the next dose.

2 Blood / infection

Chemotherapy phase

Fever, shivers or suddenly unwell: call immediately.

3 Hair / scalp

Chemotherapy phase

Hair loss is common. Ask about scalp cooling before treatment where suitable.

4 Fluid / skin / nails

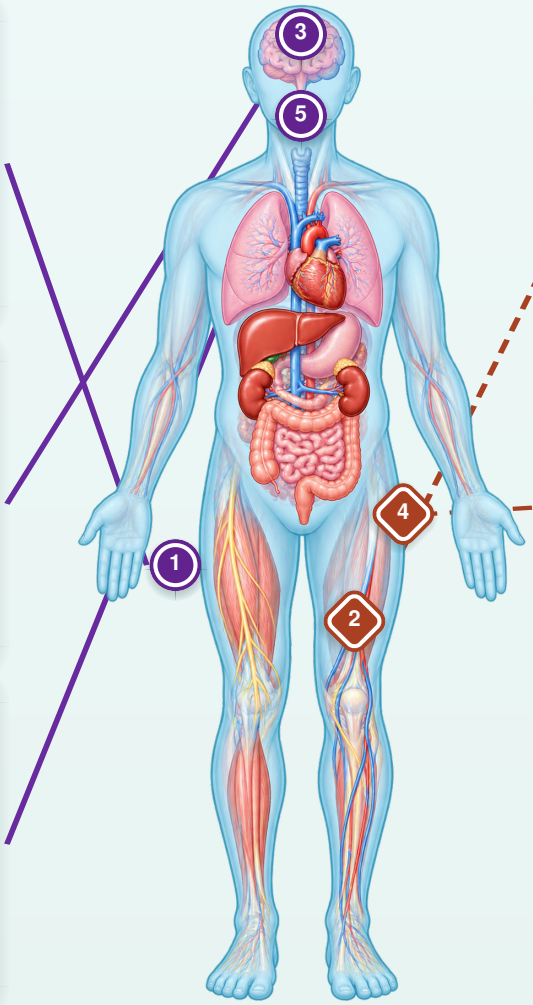
Docetaxel

Report swelling, painful skin or nail changes.

5 Mouth

Chemotherapy phase

Painful ulcers or difficulty drinking need advice.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Selected medicine-specific effects; the full risk discussion includes hair loss, fertility and other individual risks.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Docetaxel · reusable record · copy this sheet for another cycle of the same phase.

Docetaxel · Taxane chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	□ None	□ Mild change; usual tasks unaffected	□ Limits buttons, writing or walking: call	□ Washing, dressing or walking difficult: urgent review	□ Sudden major weakness / paralysis: emergency help
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow the contact plan from your own hospital.

SACTCHECK · SACT TREATMENT

Docetaxel 75 mg/m² · every 3 weeks

NCCP 00203 v7 · 21-day cycle

Docetaxel 75 mg/m² by IV infusion on Day 1 every 21 days. For the source breast indication, continue until progression or unacceptable toxicity; the team reviews benefit and tolerability. The six-cycle source maximum applies to NSCLC.

1 2 3 4 5 6

Your contact plan

Oncology team: Insert numbers here
Out of hours: Insert numbers here.

Evenings / overnight: instructed out-of-hours pathway or nearest ED.
Emergency: 112 / 999.

Other hospital / own contact: _____

QR: https://sactcheck.com/patient/00203/ · no patient information.

Draft for review · SACTCheck v0.84.21

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00203 v7 · official regimen ↗**
   https://healthservice.hse.ie/documents/6543/203_DOCEtaxel_75-21day.pdf
2. **EMA Taxotere product information**
   https://www.ema.europa.eu/en/documents/product-information/taxotere-epar-product-information_en.pdf
3. **Harvey et al. Phase III docetaxel dose comparison; JCO 2006; PMID 17033039**
   https://pubmed.ncbi.nlm.nih.gov/17033039/
4. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
5. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
6. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
7. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
8. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
9. **NICE NG 101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
10. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
11. **NCCP NCIS antiemetics v 8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
12. **ASCO · biomarkers in metastatic breast cancer**
    https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
13. **ESMO metastatic breast cancer guideline · 2026**
    https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
14. **NCCP antiemetic guidance; reconcile with regimen and individual prescription**
    https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf